June 16, 2025

The Honorable Mehmet Oz, Administrator
Centers for Medicare & Medicaid Services
Department of Health & Human Services
200 Independence Ave SW
Washington, DC 20201

Re: Request for Information: Health Technology Ecosystem

Dear Dr. Oz,

I am the Chief Executive Officer of Particle Health, Inc. ("Particle"), a healthcare technology company founded in 2018 that breaks down data silos in healthcare by transforming fragmented medical records into comprehensive, usable insights that help providers deliver better care. Our intelligent data platform connects to national and state medical record exchanges, aggregating and delivering actionable patient insights through a user-friendly API. Today Particle gives healthcare organizations access to over 320 million patients' medical records from more than 70,000 healthcare organizations.

Particle appreciates the opportunity to provide feedback on the Centers for Medicare & Medicaid Services (CMS) Request for Information regarding the digital health ecosystem, with a focus on interoperability. As a company at the intersection of healthcare and technology, we have firsthand experience with the challenges and opportunities in the current digital health ecosystem. Our response addresses the specific questions posed in the RFI, drawing on our expertise and experience in the field.

As we discuss more fully below, the following points are critical to advancing true healthcare interoperability:

- **Enforce existing information blocking rules through a proactive, centralized entity.** Information blocking by large health IT entities creates information monopolies and fragments patient care through deliberate tactics, including non-responsiveness and unusable responses, with QHINs acting as chokepoints that can control data flow and limit competition. RCEs should have responsibility for QHIN oversight and establish clear, enforceable compliance designed to eliminate information blocking.
- **Bar QHINs from imposing excessive administration requirements that do not demonstrably enhance patient privacy or data security**. Administrative requirements imposed by QHINs create unnecessary friction for Integrated Delivery Networks and smaller providers, diverting resources from patient care to compliance paperwork and potentially causing small practices to disengage from health information exchange altogether.
- **Mandate clear timelines and technical requirements for responding to operational and IAS queries via TEFCA, with associated penalties and enforcement.** TEFCA's operational and patient access provisions are promising but require stronger

implementation, including a fixed date by which responding to operational queries becomes mandatory with no fees and clear technical requirements that allow patients to opt in to IAS with standardized patient identity verification and access at little or no cost.

I. Enforce Existing Information Blocking Rules Through A Proactive, Centralized Entity [PC13, PR-12, PR-14, TD-18]

Particle Health has observed multiple forms of information blocking as part of its participation in nationwide health information exchange networks. These include both non-responsiveness and responses that are designed to fail or be unusable. Specific examples of non-responsiveness include failure to respond to queries despite confirmation that patient data exists; excessive delays in response times without technical or legal justification; and selective denials of Particle Health queries. Examples of responses that are designed to fail include providing incomplete clinical records or records that lack any clinical data–effectively rendering the response useless, and returning documents that are non-standardized, corrupted, or improperly coded, which break interoperability. These practices materially frustrate the ability of authorized actors to access and use health information in a way that is "technically feasible and legally permissible," in violation of the intent of the Information Blocking Rule under 45 CFR Part 171.

The power to block the transfer of information in this way stems from TEFCA's structure, where influence is concentrated in QHINs. Presently, QHINs form TEFCA's backbone and consist of large networks (often EHR vendors, HIEs or major health IT organizations) that aggregate and route data for the participants. Smaller entities must join as participants or subparticipants under a QHIN, not directly with TEFCA. QHINs, in turn, exert enormous control by setting technical and operational requirements, fee structures, data routing and access, technical requirements, and certification and compliance enforcement. Aside from the potential (and incentive) to limit competition that this power creates, it also enables information blocking because a QHIN can act as a chokepoint. A QHIN that is managed by an EHR vendor may have incentives to prioritize its own customers' data flow. If a QHIN chooses to delay, restrict, or technically limit certain types of data exchange, downstream participants have limited recourse. **When a QHIN is the arbiter of compliance behavior, it can choose to ignore information blocking behaviors**. See TEFCA Common Agreement v.2.0, Section 2.1,2.1; ONC TEFCA QHIN Technical Framework, July 2024.

A case in point is Epic's exertion of control over Carequality. Epic was a founding member and primary architect of Carequality's technical framework and governance. As the largest EHR vendor in the U.S., Epic's representatives have held key positions on Carequality's steering committees and working groups, giving them significant influence over technical standards, use case prioritization, and policy decisions. Epic historically has required that any EHR vendor or participant connecting to Carequality must comply with Epic-driven technical and operational requirements. For example, Epic has pushed for document-based exchange (C-CDA) as the default, which aligns with its own architecture and makes it harder for API-first or FHIR-native vendors to participate on equal footing. Epic has, at times, limited or delayed certain types of data exchange through Carequality, citing privacy, security, or other concerns. This has allowed

Epic to control the pace and scope of interoperability, to the detriment of smaller implementers such as Particle. And, because so many hospitals and health systems use Epic, and because Epic's customers are automatically connected to Carequality, Epic can effectively dictate terms for participation. Smaller EHRs, HIEs, and API vendors often have to accept Epic's preferred policies to gain access to the network.

The non-responsive endpoints and selective disclosure resulting from the power granted to EHR vendors and health systems are not technical glitches–they are deliberate blocking tactics that require mandatory monitoring and public reporting, coupled with enforcement of the 21st Century CURES Acts' prescribed civil money penalties. To address information blocking, and to ensure that TEFCA delivers on its promise, the following four structural changes and enforcement mechanisms should be implemented:

- **The RCE should enact direct, centralized oversight of QHINs.** Today there is a delegated enforcement model where QHIN participants self-police, with responsibility to identify and enforce information blocking claims pushed downstream to participants. This distributed framework creates accountability gaps as QHINs lack standardized monitoring requirements, allowing non-responsive endpoints and selective disclosure practices to persist without consequence. As a result, participants need to monitor unexplained denials, timeouts, or empty responses to treatment queries and, thereafter, attempt to cajole the blocking participant to stop its conduct. If the blocking participant refuses to do so, the monitoring participant's only recourse is to file a dispute, draining its legal and financial resources in the process. This is not an effective enforcement mechanism.[1]
- To avoid plausible deniability by entities that engage in information blocking, the **RCE should clarify permissible vs prohibited practices**. CMS or the RCE should issue more detailed sub-regulatory guidance or FAQs that illustrate common blocking scenarios, such as selective implementer denial, excessive response delays, and provision of non-standardized documents to establish clear and incontrovertible rules of the road.
- To replace the current patchwork of inconsistent participant-level reporting, the **RCE should implement systematic auditing** for selective disclosure behaviors through cross-network pattern analysis that QHINs themselves have little incentive to identify;
- To eliminate the implementation variations that participants currently permit, t**he RCE should mandate enforceable remediation timelines,**

The mechanisms will establish clear consequences for non-compliance, including potential suspension of QHIN status, creating powerful incentives for all networks to prioritize complete,

[1] QHINs also should be required to have more inclusive governance and lower barriers for API-based innovation. QHINs must offer transparent, non-discriminatory access and pricing, and governance representation should include more small vendors, API providers, and patient advocates.

timely responses regardless of the requesting entity's affiliation. By consolidating these oversight functions within the RCE, enforcement becomes consistent and impartial.[2]

Finally, **information blocking enforcement should be public, predictable, and deterrent.** To date, enforcement of information blocking has been limited, opaque, and slow. This emboldens bad actors and discourages smaller or more innovative participants from investing in compliant, interoperable solutions. Starting today, CMS and the HHS OIG should aggressively investigate information blocking claims and impose, where warranted, the muscular penalties prescribed in the 21st Century CURES Act. Going forward, TEFCA should explicitly prohibit actors from preferentially responding to certain networks, vendors, or implementers. Enforcement should include monitoring for differential response patterns and penalties for discriminatory practices. The RCE should continuously coordinate with the ONC to share the data that it has gathered supporting an information blocking claim. Without enforcement, data hoarding will continue to benefit entrenched incumbents and block emerging use cases—especially those dependent on agentic workflows, cross-vendor analytics, and patient-directed access. Robust enforcement will create the conditions necessary for smaller providers and innovators to compete on quality, not data access; patients to experience continuity of care across health systems; and rural and underserved communities to access the same digital tools and coordinated care as well-resourced urban areas.

II. Bar QHINs from Imposing Excessive Administration Requirements That Do Not Demonstrably Enhance Patient Privacy Or Data Security [PR-6, PR-7, PR-8 PC-10, PC-11, PA-1].

TEFCA has made significant progress in establishing the infrastructure necessary for nationwide health information exchange. Several networks have achieved QHIN designation and begun exchanging data under the Common Agreement, representing a critical milestone in enabling cross-organizational treatment queries. The framework provides standardized exchange policies and technical requirements, reducing legal uncertainty that previously impeded adoption.

TEFCA strategically bridges existing frameworks and future infrastructure by codifying treatment-based exchange as a baseline purpose of use. This effort to align with HIPAA and Carequality creates a foundation for broader interoperability while maintaining continuity with established exchange patterns. While continuing this national effort, **we must also create policies that drive alignment between TEFCA's national framework and the regional expertise of Health Information Exchanges ("HIEs"), with their established trusted relationships with local healthcare organizations.** Doing so will significantly enhance

[2] To the extent that CMS seeks to place the burden of reporting instances of information blocking onto patients, we respectfully disagree with that approach. Patients should have the ability to report suspected instances of information blocking, but the responsibility to prevent and remedy information blocking lies with those who have the tools to detect it in real time QHINs, health IT developers, and health care providers. To make reporting more accessible to patients, CMS could educate patients on their rights and include links to ONC's Information Blocking Complaint Portal in channels that patients already access, such as patient portals.

healthcare interoperability. Local HIEs have developed deep relationships with local healthcare organizations and understand regional needs, workflows and challenges. This granular knowledge complements TEFCA's national standardization approach, allowing HIEs to help local organizations implement and optimize their participation in the broader TEFCA framework, potentially as local QHIN participants that facilitate connections between local providers and the national framework. This role would leverage HIEs' existing technical infrastructure while extending their reach through TEFCA's network-of-networks approach.

TEFCA's provider access benefits are real but not yet universal. **Without faster technical convergence, fee transparency, and onboarding simplification, many providers will continue to rely on legacy networks and duplicative connections.** Currently, providers can retrieve patient records from a broad array of participating organizations and QHINs, which reduces the need for multiple point-to-point connections or memberships in various regional HIEs. Integration with federal agencies, such as the Social Security Administration and the Department of Veterans Affairs, further reinforces provider access by incorporating government-held health data into routine clinical workflows.

Moreover, TEFCA's patient access provisions are promising but not yet fully realized. Without a narrower ability to sidestep the mandate, technical enforcement, and patient-centric design, actual patient empowerment may lag behind policy intent. Consequently, Particle urges that CMS and the RCE require participants to conspicuously allow patients to opt in to the Individual Access Services ("IAS") function for all QHIN's and participants, with clear technical and workflow requirements. Patient access requests should be at little or no cost to patients, and QHINs should be be required to publish clear, patient-friendly instructions and metrics on IAS availability and performance. To reduce friction and false negatives, QHINs should standardize and simplify patient identity verification. Patients currently have to manage multiple logins and portals; TEFCA-backed aggregation could streamline this.

Several improvements would enhance TEFCA's impact. **Streamlining and standardizing** the onboarding process would lower barriers for smaller practices and independent providers. **Greater transparency and caps on QHIN-imposed fees** would help ensure equitable participation and prevent financial barriers to access. Expanding the required exchange purposes to include care coordination, quality reporting, and value-based care analytics would further align TEFCA with evolving clinical and regulatory needs.

Importantly, **requiring QHIN's to permit Integrated Delivery Networks ("IDN's") of all kinds, including Accountable Care Organizations and Clinically Integrated Networks, to query as "Principals" would significantly reduce administrative friction on smaller and rural providers.** IDNs exist to support seamless care across affiliated providers, often operating under centralized governance and shared clinical infrastructure. Where QHIN's impose rigid, one-size-fits-all delegation models that require an IDN to obtain Delegation of Authority notices from each practice within the IDN, this carefully calibrated care coordination facilitated by the IDN is disrupted. Rather than facilitating interoperability, these requirements risk fragmenting networks and introducing friction into the exchange of data between affiliated entities—particularly when existing governance required by management services agreements and business associate agreements already ensures aligned compliance and oversight. The

administrative burden will divert clinicians and staff from patient engagement to focus on compliance and paperwork—putting timely, effective treatment at risk for vulnerable populations. Clinicians may be pulled away from critical patient interactions, small practices may opt out of interoperability altogether, and time-sensitive data exchange may falter, putting patient safety and care quality at risk. These challenges are compounded for smaller and rural providers within IDNs. These organizations typically lack the legal and administrative resources to navigate new documentation obligations and will be disproportionately burdened by the Delegation of Authority policy's complexity. The practical effect may be a chilling one—smaller practices could disengage from QHINs or restrict their participation, which directly harms the patients who most need coordinated access to their health data. If practices are forced to disengage due to administrative burden, the real harm is to the patients, many of whom are vulnerable, who depend on integrated systems to receive accurate, efficient care.

III. Mandate Clear Timelines And Technical Requirements For Responding To Queries Via TEFCA, With Associated Penalties And Enforcement [PA-2, PA-5, PA-7]

TEFCA's lack of clear, mandatory requirements for responding to operational and IAS queries is a critical gap that undermines its effectiveness. WIthout explicit obligations compelling QHINs and their participants to respond to these types of requests, the framework allows for discretionary and inconsistent participation, resulting in unpredictable data availability for both providers and patients. This ambiguity enables network participants to deprioritize or delay responses to operational queries - such as those needed for care coordination, quality reporting, or population health management - as well as IAS queries initiated by patients seeking access to their own health information. The absence of a uniform mandate perpetuates the current fragmentation, as some entities may fully support these queries while others do not, leading to gaps in information flow and undermining the promise of seamless interoperability.

In addition, enforceable operational query response requirements are critically important because TEFCA currently does not require a response to queries for HIPAA Treatment ("TREAT") while mandating a response to TEFCA Required Treatment "TRTMNT". The TRTMNT purpose of use excludes many essential healthcare healthcare coordination activities that must, instead, rely on operational queries. This limitation particularly undermines value-based care models, as risk-bearing entities cannot effectively manage population health when cross-organizational data exchange for operational purposes remains discretionary. Further complicating matters, unlike treatment queries that must be provided at no cost, entities can charge potentially prohibitive fees for operational query responses, creating financial barriers to comprehensive interoperability.

The absence of enforceable operational query response requirements particularly undermines the data foundation upon which value-based care models depend, creating an urgent need for a fixed compliance date that healthcare organizations can incorporate into their strategic planning. Risk bearing entities, such as Accountable Care Organizations and other IDNs, cannot effectively manage population health when cross-organizational data exchange for operational purposes remains unreliable and discretionary. The impact on longitudinal care coordination is particularly acute. Chronic disease management protocols require consistent monitoring of lab values, medication adherence, and vital signs across

disparate providers. This monitoring fails when data exchange for operational purposes remains optional rather than mandatory, driving adverse health outcomes.[3] Moreover, research demonstrates that care fragmentation disproportionately affects vulnerable populations, including those who are low-income or uninsured or non-white.[4]

Imposing a timeline for QHIN's to require mandatory, timely responses to both operational and IAS queries would establish a consistent baseline for data exchange, significantly reducing variability and uncertainty across the network. Such a mandate would ensure that providers can reliably obtain the information necessary for efficient clinical operations and that patients can exercise their right to access their health data without encountering arbitrary barriers. Until TEFCA operationalizes these requirements, the framework will continue to fall short of its goal to unify the nation's health information infrastructure, leaving both patients and providers exposed to the inefficiencies and risks of a fragmented exchange environment.

Sincerely,

Jason Prestinario
Chief Executive Officer
Particle Health. Inc.

[3] https://pmc.ncbi.nlm.nih.gov/articles/PMC10170908/
[4] https://pmc.ncbi.nlm.nih.gov/articles/PMC8045386/